



Interview Room Use - Orchard Avenue Centre Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Mental Health Service	Orchard Avenue Centre	All disciplines

2. Purpose

The purpose of this procedure is to provide a safe and secure environment for all patients, visitors, and staff of Armadale Mental Health Service (AMHS). Interviewing Mental Health consumers is a situation where employees are potentially at risk of injury or harm to their health should violence or aggression occur. This procedure outlines requirements and strategies which will assist in mitigating the risk of harm/injury to clinicians should a consumer/family member/carer become aggressive or violent before or during an interview.

This procedure should be read in conjunction with, WA Health Clinical Care of People with Mental Health Problems Who May be At Risk of Becoming Violent or Aggressive Policy MP 0101/18; Clinical Risk Assessment and Management (CRAM) in Western Australia Mental Health Services; and Code Black Response – Orchard Avenue Centre.

3. Procedure

3.1 Terminology

Low-Risk Consumer	Consumer with a known mental health history and no evidence of aggression or violence and assessed as low risk on risk assessment.
Medium-Risk Consumer	A consumer with a known history of low-level aggression or alerts in their history but this is not evident in their current presentation.
High-Risk Consumer	A consumer who is believed at the time of presentation to have the propensity to commit violence or is aroused, disturbed or agitated; any patient who has a history of medium to high-risk ratings for aggression/violence.

3.2 Assessment preparation

Consumers attending the centre without appointments, not known to the mental health service or active with known risk management plans where there is a risk of aggression, are required to be screened by clinical staff to identify risk and safely manage their visit to the clinic.

On presentation to centre reception having identified that contact with a clinician is required, reception staff will invite the consumer to take a seat in the waiting area.

Reception staff will inform ATT or Older Adult Service Triage Clinicians of a consumer presentation, or in the case of active clients the consumers Care Co-ordinator. If the Care Co-ordinator is not available ATT Triage will be notified for ATT consumers; First on Call (FOC) for CTT consumers; and Older Adult Triage for OA consumers.

Clinical staff will invite the consumer to take a seat at the triage window in reception.

The triage window is to be utilised to provide a semi-private area for consumers when interacting with clinicians slightly aside from the main waiting area. It is designed to facilitate a brief interaction with a consumer to determine their presentation and mental state before they enter the main clinical areas of the centre for treatment or assessment.

Clinical staff will conduct a risk assessment (WebPSOLIS RAMP) which will determine the most suitable venue for further assessment. The staff member conducting the interview will review the following to inform their assessment of the consumer's risk for violence/aggression:

- Most recent risk assessment in the medical record and/or Psychiatric Online Information System (PSOLIS).
- PSOLIS alerts or receive handover regarding PSOLIS alerts.
- Recent progress notes to gauge mental state and any significant changes indicating an increased level of risk.
- Current mental state and presentation.

Where it is identified that a consumer may present a behavioural risk to staff, further assessment should occur in the Medium-Risk or High-Risk interview rooms.

Where it is determined the consumer does not present a behavioural risk to self or others but would benefit from a quiet environment prior to further assessment/interview, the Low-Stimulus room may be utilised. Inclusion criteria for use of the Low-Stimulus room requires that consumers are assessed as being able to self-regulate their emotions in a low-stimulus environment; reduce anxiety and agitation; and prevent behaviours leading to behavioural escalation. The consumer must be assessed as safe to leave unsupervised in the Low-Stimulus room for no longer than 15 minutes, at which time they will be re-assessed to determine level of risk. The Low-Stimulus room is unsuitable for High-Risk consumers requiring a high level of supervision.

Consumers who are assessed as low risk may be invited to have their assessment in a standard interview room.

Consumers attending for a regular appointment will have this scheduled in advance by clerical staff and this will be conducted in a standard interview room unless the consumer presents with risks that would indicate another interview room type would be more appropriate.

3.3 Interview room function

When utilising the any type of interview room clinicians will ensure that:

- The consumer is seated nearest the door they entered the room.
- Staff are positioned nearest to the staff egress exit door.
- There is adequate spacing between the clinician and the patient.
- Staff have a duress pendant/alarm readily available as required.
- The consumer is observed for signs of distress or increased levels of agitation and arousal.

- If required, tactful, non-confrontational intervention/de-escalation techniques are utilised to diffuse a situation.
- The duress is activated if feeling threatened and clinician removes themselves to a staff only safety area.

3.3.1 Standard interview room (x14)

The room is enclosed with indirect access from the waiting area. The room contains solid, comfortable furniture for the consumer that is not easily manoeuvrable. Security features include fixed duress point; CCTV in corridors; emergency access control on the egress door leading to a safe corridor with a safe locking mechanism to prevent consumer following the clinician into the safe corridor.

3.3.2 Medium-risk interview room (x1):

Room is enclosed with indirect access from waiting area, featuring an extra wide desk with an adjacent glazed door between the consumer and clinician, to reduce the risk of interpersonal violence and provide staff with additional time to exit the room if required. Security features include fixed duress points; CCTV in corridors; swipe access on the internal door between consumer and clinician; emergency access control on the egress door leading to a safe corridor with a safe locking mechanism to prevent consumer following the clinician into the safe corridor.

3.3.3 High-Risk interview room (x1):

The room is enclosed with direct access from the waiting area. Functionally identical to Medium-Risk room with the addition of a full height safety glass barrier in the middle of the desk. The entry door to this room can be remotely unlocked by clerical staff in reception on instruction from clinical staff. Security features include fixed duress points; CCTV in corridors; swipe access on the door between consumer and clinician; emergency access control on the egress door leading to a safe corridor with a safe locking mechanism to prevent consumer following the clinician into the safe corridor.

3.3.4 Low-stimulus room (x1)

The room is enclosed with direct access from the waiting area, with functional ergonomics to facilitate low sensory stimulation. The entry door to the room can be remotely unlocked by clerical staff in reception on instruction from clinical staff.

3.4 Post high risk interview

- Clinicians will inform the treating team of the completion of any High-Risk interview and the outcome.
- The outcome of the interview and any changes to the consumer's risk assessment and/or overall management plan will be documented in the patient's medical record and PSOLIS and discussed with the multidisciplinary team.
- If any escalation or an actual/threatened aggressive/violent incident has occurred during the interview the staff members involved will:
 - Complete Combined Hazard or Incident Reporting (CHoIR).
 - Complete Datix Clinical Incident Management System if appropriate.
 - Add a new PSOLIS alert if clinically indicated.

Senior staff will:

- Ensure any person affected by the incident is safe and that the necessary steps are taken to support and treat persons involved.
- Make the surroundings are safe to prevent immediate recurrence of the incident.
- Arrange appropriate (individual/group) debrief for all staff involved in the incident.
- Inform staff of the availability of the Employee Assistance Program.

4. Related Policy and Procedure Documents

- [WA Health Clinical Care of People with Mental Health Problems Who May be At Risk of Becoming Violent or Aggressive Policy MP 0101/18](#)
- Code Black Response – Orchard Avenue centre

5. Relevant standards

National Safety and Quality Health Service Standards – second edition

- Standard 5: Action 5.13 Developing the comprehensive care plan

6. Legislative Context

The following documents are required to give effect to this procedure:

- *WA Mental Health Act 2014*
- *Criminal Code Act 1913*
- *Occupational Safety and Health Act 1984*

7. References

- [WA Health State-wide Standardised Clinical Documentation SSCD for Mental Health Services OD 0526/14](#)
- [Chief Psychiatrist of Western Australia: Reporting Notifiable Incidents – Public Mental Health Services](#)
- [Clinical Incident Management Policy MP 0122/19](#)
- [East Metropolitan Health Service \(EMHS\) Prevention and Management of Workplace Aggression and Violence](#)
- [EMHS Duress Alarm Systems](#)
- [EMHS OSH: Incident and Hazard Reporting](#)
- [AKG Risk Management Procedure](#)
- [AKG Damage to Property and Equipment](#)

8. Policy Owner (relating to these procedures)

Shannon Stuart – Program Manager Community Mental Health

Enquiries relating to this procedure may be directed to:

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9. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	18/06/2025	18/06/2028	Scheduled Review

10. Authorisation

This procedure has been authorised and issued by Service Director Mental Health:

Authorised by	Karen Elliott - Service Director Mental Health
Authorisation date	27/05/2025
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